WEEK 3 TASK

Regulatory Document Drafting for Clinical Research

Clinical Trial Protocol Summary

Protocol title	A Randomized Controlled Study of Structured Lifestyle Counselling Plus Usual Care Versus Usual Care Alone in Adults with Obesity
Protocol type	Hypothetical educational protocol summary
Design	Multicentre, parallel-group, randomized controlled study; blinded outcome assessment where feasible
Population	Adults aged 18–65 years with BMI ≥30 kg/m², subject to final eligibility criteria
Duration	24 weeks per participant
Primary endpoint	Percentage change in body weight from baseline to Week 24
Framework	ICH GCP; applicable Indian requirements including NDCTR 2019, CDSCO, Ethics Committee and trial registration requirements

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Important: This is a hypothetical educational draft, not an approved protocol or permission to conduct research. A real study requires review and approval by qualified professionals and relevant authorities.

Protocol Synopsis

This document outlines the study rationale, objectives, design, eligibility, procedures, statistical approach, safety oversight, data management, regulatory safeguards, deviation handling and reporting plan.

Detailed Protocol Summary

1. Introduction

Clinical research generates evidence about the safety, effectiveness and appropriate use of interventions in people. A clinical trial protocol is the operational and scientific plan for a study. It explains why the research is needed, what will be done, who may participate, how outcomes will be measured, how participant safety will be protected and how the results will be analysed. A clear protocol supports consistent conduct across sites, ethical review, regulatory assessment and reliable interpretation of findings.

This document presents an educational, hypothetical protocol summary for a randomized study of structured lifestyle counselling in adults with obesity. Obesity is a chronic, multifactorial condition associated with increased risk of type 2 diabetes, cardiovascular disease, sleep apnoea and other health problems. Standard care may include advice on nutrition, physical activity and behavioural change, but the intensity and consistency of support can vary. A structured programme may improve adherence by providing planned counselling, practical goals and regular follow-up.

The proposed study compares structured lifestyle counselling plus usual care with usual care alone over 24 weeks. It is designed to evaluate change in body weight and selected metabolic and behavioural outcomes. The study is not intended to replace clinical judgement or establish a treatment recommendation by itself.

The protocol is informed by the principles of Good Clinical Practice (GCP), including the current ICH E6(R3) principles and Annex 1, and by applicable Indian requirements, particularly the New Drugs and Clinical Trials Rules, 2019 (NDCTR), where relevant.

2. Study Objectives and Endpoints

Primary objective: To compare the effect of structured lifestyle counselling plus usual care against usual care alone on percentage change in body weight from baseline to Week 24 in adults with obesity.

Primary endpoint: Percentage change in body weight at Week 24, calculated as [(Week 24 weight – baseline weight) / baseline weight] × 100. Weight will be measured using a calibrated digital scale, with participants wearing light clothing and no shoes, following a standardised procedure.

Secondary objectives are to compare the groups in: (1) change in waist circumference at Week 24; (2) change in systolic and diastolic blood pressure; (3) change in fasting glucose and lipid profile, where clinically appropriate and consented; (4) change in self-reported physical activity and dietary behaviours; (5) attendance and adherence to counselling sessions; and (6) safety and tolerability, including events that may be related to dietary or activity changes.

Exploratory objectives may include participant satisfaction, perceived barriers to behaviour change and maintenance of weight change at a later follow-up, if resources permit. Exploratory outcomes will be labelled as such and will not be treated as definitive evidence.

The primary endpoint, time point and statistical method will be specified before the database is locked.

3. Study Design and Setting

This is a hypothetical, multicentre, parallel-group, randomized, controlled, open-label study with blinded outcome assessment where feasible. Eligible participants will be assigned in a 1:1 ratio to structured lifestyle counselling plus usual care or usual care alone. An open-label design is practical because participants and counsellors will know whether counselling is provided; however, staff measuring weight and analysing coded data should remain unaware of group allocation when feasible.

The proposed study duration for each participant is 24 weeks, with screening and baseline procedures completed before randomization. Recruitment and follow-up may take approximately 9–12 months depending on site capacity. The number of participating sites and the final sample size must be confirmed during feasibility assessment. The study will be conducted at qualified outpatient or clinical research sites with appropriate facilities, trained staff and access to medical review.

The control group will continue to receive usual care available at the site. The intervention group will receive usual care plus a structured programme delivered by trained personnel. The programme may include an initial individual session, scheduled follow-up counselling, goal setting, nutrition education, physical-activity planning and behaviour-change support.

4. Study Population and Eligibility

The target population is adults receiving care for obesity. Potential participants will be identified through approved clinic referrals, advertisements or other Ethics Committee-approved recruitment methods. Recruitment materials will be factual, non-coercive and free from promises of benefit. Clinical staff will explain that participation is voluntary and will not affect access to ordinary care.

Proposed inclusion criteria are: (1) age 18–65 years; (2) body mass index (BMI) of at least 30 kg/m², using a prespecified measurement method; (3) willingness and ability to attend study visits and follow the study procedures; (4) ability to provide informed consent; and (5) agreement to permit collection and use of study data as described in the consent documents.

Proposed exclusion criteria are: (1) pregnancy or breastfeeding, where the study procedures or weight-management intervention may be unsuitable; (2) a medical condition that makes participation or the proposed activity plan unsafe, as judged by a qualified clinician; (3) recent major weight-loss surgery or planned bariatric surgery during the study; (4) current participation in another interventional study that could affect outcomes; (5) recent initiation of a weight-management medicine or programme likely to confound assessment, according to a prespecified interval; or (6) any other condition that, in the investigator’s judgement, creates an unacceptable risk or makes reliable participation unlikely.

5. Recruitment, Consent and Study Procedures

At screening, authorised study staff will explain the study in a language the potential participant understands, provide the participant information sheet and allow adequate time for questions and consideration. Consent must be obtained before any study-specific procedure. The person obtaining consent will confirm understanding, assess voluntariness and document the process. A participant who cannot read should receive an appropriate impartial witness-assisted process consistent with applicable requirements. The participant will receive a copy of the signed consent form.

Screening procedures will include review of medical history, current medicines, relevant physical measurements and eligibility criteria. Baseline assessments will include weight, height, BMI, waist circumference, blood pressure and agreed questionnaire measures. Laboratory tests, if included, will be collected by qualified personnel using validated procedures and appropriate sample handling. Participants will then be randomized and given their group-specific instructions.

Suggested visits are baseline, Weeks 4, 8, 12, 18 and 24, with a permitted visit window specified in the detailed protocol. At follow-up visits, staff will assess body weight, adherence, changes in medicines or health status, adverse events and any need for referral. The intervention group will receive the planned counselling sessions.

Missed visits will prompt a respectful follow-up using contact methods agreed in the consent form.

6. Sample Size and Statistical Analysis

The sample size should be calculated from the expected between-group difference in percentage weight change, its variability, the chosen type I error rate and statistical power. These assumptions must be supported by published evidence or pilot data, and the calculation should be reviewed by a qualified statistician. An allowance for anticipated attrition should be added. Because reliable estimates are not supplied in this educational example, no definitive sample size is claimed; the final number must be justified before protocol approval.

The primary analysis will compare percentage change in body weight from baseline to Week 24 between randomized groups. A suitable model may use analysis of covariance (ANCOVA), with treatment group as a fixed effect and baseline weight and prespecified stratification factors as covariates, if assumptions are reasonable. The analysis plan will define the analysis population, missing-data approach, handling of protocol deviations, outlier checks and sensitivity analyses. The primary analysis should follow the intention-to-treat principle, including participants in the groups to which they were randomized, as appropriate to the estimand and available data.

Secondary outcomes will be analysed using methods appropriate to the type and distribution of data.

7. Safety Monitoring and Risk Management

The intervention is behavioural, but participation may involve inconvenience, emotional discomfort, musculoskeletal symptoms from increased activity or risks associated with changes in diet. Risks will be minimised through individual assessment, gradual goal setting, trained counselling staff and clear instructions to stop an activity and seek advice if concerning symptoms occur. Participants will not be asked to stop prescribed medicines or delay necessary medical care.

An adverse event (AE) is any unfavourable medical occurrence during the study, whether or not it is considered related to participation. A serious adverse event (SAE) is an event meeting applicable seriousness criteria, such as death, a life-threatening event, hospitalisation or significant disability. Investigators will assess severity, seriousness, expectedness where relevant and possible relationship to study procedures. Events will be documented and reported to the sponsor and Ethics Committee within the timelines required by applicable law and the approved safety plan.

A qualified investigator will review clinically significant findings and arrange referral or treatment when needed. The protocol will specify criteria for temporary interruption, permanent discontinuation, urgent medical evaluation and withdrawal for safety. The sponsor and investigator will periodically review emerging safety information.

Participants will be told how to contact the study team and what to do in an emergency.

8. Data Management, Confidentiality and Quality Assurance

Each participant will be assigned a unique study identification code. Direct identifiers will be stored separately from study data, with access limited to authorised personnel. Electronic records will use individual accounts, role-based access, secure storage, backups and audit trails where supported. Paper records will be kept in locked, access-controlled locations. Data handling will follow applicable privacy and data-protection requirements.

A data management plan will describe source records, case report forms, data entry, coding, edit checks, query resolution, database locking, retention and controlled access. Data will be checked for completeness, consistency, plausibility and protocol-defined ranges. Corrections will be traceable, dated and attributable; original entries will not be obscured. Only authorised staff may make changes, and queries should be resolved before database lock. The statistical analysis plan will be finalised before unblinded analysis.

Quality management will focus on factors critical to participant protection and reliable results. Training records, delegation logs, consent records, screening logs, accountability for study materials and protocol deviations will be maintained as appropriate. Monitoring may be on-site, remote or centralised according to risk and applicable requirements. Audits or inspections may be conducted by authorised parties.

9. Regulatory and Ethical Considerations

The study will follow ethical principles for research involving humans, applicable national laws and Good Clinical Practice. ICH E6(R3) emphasises participant rights, safety and well-being, reliable results, clear responsibilities and quality built into study design. The protocol should also reflect the principles of ICH E8(R1), including a study design that is fit for purpose and proportionate to risk.

In India, the sponsor must determine whether CDSCO permission is required for the specific study and intervention under the New Drugs and Clinical Trials Rules, 2019, as amended. An appropriately constituted and registered Ethics Committee must review the protocol, informed consent documents, recruitment materials, investigator qualifications and relevant participant protections before initiation. No participant may be enrolled before all required approvals are in place. Changes to the protocol or consent documents must undergo the required prior review and approval, except urgent measures necessary to remove an immediate hazard, which must be reported as required. The approved version and amendment history will be controlled.

The consent process will explain the study purpose, procedures, duration, foreseeable risks and potential benefits, alternatives, confidentiality limits, compensation or treatment arrangements where applicable, contacts for questions and the right to refuse or withdraw without penalty. Participation will not be described as guaranteed personal treatment or guaranteed benefit.

10. Protocol Deviations, Amendments and Early Termination

A protocol deviation is a departure from the approved protocol or study procedures. Deviations will be documented with the date, description, reason, impact on participant safety and data reliability, and corrective or preventive action where needed. Significant deviations will be escalated to the sponsor and Ethics Committee according to the approved procedures. Repeated deviations will trigger a review of training, feasibility and site processes.

Substantial changes to eligibility criteria, study design, endpoints, procedures or consent information must not be implemented until required sponsor, Ethics Committee and regulatory approvals have been obtained. A change needed immediately to eliminate an urgent hazard may be implemented as legally permitted, followed by prompt notification and documentation. Version control will ensure staff use only current approved documents.

The sponsor may suspend or terminate the study for safety concerns, insufficient recruitment, serious non-compliance, unreliable data or other justified reasons.

11. Reporting, Dissemination and Conclusion

After database lock and analysis, the sponsor and investigators will prepare a clinical study report or appropriate final study report describing methods, participant flow, baseline characteristics, outcomes, safety observations, protocol deviations and limitations. Results will be presented transparently and will not be selectively reported to favour the intervention. Authorship and acknowledgement will follow recognised publication standards, and any public communication will remain consistent with the analysed evidence.

The proposed protocol provides a structured framework for assessing whether a planned lifestyle-counselling programme improves weight-related outcomes over 24 weeks compared with usual care. Its strengths are a defined primary endpoint, random allocation, standardised measurement, planned data management and explicit participant protections. Important limitations include the open-label nature of the intervention, reliance on some self-reported behaviours, possible loss to follow-up and the fact that a single study may not establish long-term effectiveness.

In conclusion, a high-quality clinical trial protocol must combine a scientifically answerable question with practical procedures, valid analysis, transparent reporting and protection of participants. Regulatory requirements must be checked for the specific study rather than assumed from a generic template.

References

Official sources used to describe the regulatory framework. Always verify the current requirements for the actual study and jurisdiction.

1. International Council for Harmonisation (ICH). ICH E6(R3) Guideline for Good Clinical Practice: Principles and Annex 1. Current version effective 23 July 2025.
   https://www.ema.europa.eu/en/scientific-guidelines/ich-e6-good-clinical-practice

2. Central Drugs Standard Control Organisation (CDSCO), Government of India. The New Drugs and Clinical Trials Rules, 2019, as amended. Official regulatory resources.
   https://cdsco.gov.in/opencms/opencms/en/Acts-and-rules/New-Drugs/

3. CDSCO, Government of India. Ethics Committee: responsibilities and regulatory information under NDCTR 2019.
   https://cdsco.gov.in/opencms/opencms/en/Clinical-Trial/Ethics-Committee/

4. World Health Organization (WHO). WHO Trial Registration Data Set (TRDS), Version 1.3.1.
   https://www.who.int/tools/clinical-trials-registry-platform/network/who-data-set

5. World Health Organization (WHO). Clinical trials: questions and answers, including the importance of trial registration.
   https://www.who.int/news-room/questions-and-answers/item/clinical-trials

6. International Council for Harmonisation (ICH). E8(R1) General Considerations for Clinical Studies. Refer to the official ICH/EMA guideline collection.
   https://www.ema.europa.eu/en/ich-e8-general-considerations-clinical-studies-scientific-guideline

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